

Clinical Progress Note

Patient: Jane Doe Member ID: MBR-778241 Provider: Dr. A. Reyes
Date of Visit: 2026-06-02 CPT Code: 72148 ICD-10: M54.16

Subjective: Patient presents with a six week history of lower back pain radiating into the left leg, consistent with lumbar radiculopathy. Pain is rated 7/10, worsened by prolonged sitting and improved with rest.

The patient has completed a structured course of physical therapy consisting of twice-weekly sessions over the past six weeks, including core stabilization exercises, stretching, and manual therapy, with only marginal improvement in symptoms. Given the persistence of pain despite adequate conservative

management, advanced imaging is recommended to rule out a structural cause such as disc herniation prior to considering further intervention.

No red-flag symptoms are present: no bowel or bladder dysfunction, no saddle anesthesia, no progressive motor deficit.

Assessment: Lumbar radiculopathy, unresponsive to six weeks of conservative therapy. Medical necessity for non-contrast MRI lumbar spine is established given persistent symptoms despite guideline-concordant conservative management.

Plan: Refer for MRI lumbar spine without contrast (CPT 72148).